Welcome To The THINNER Weight Loss Protocol Clinical Training

The THINNER Weight Loss Protocol uses a 2-pillar approach that is based on the 4 best practices of successful long-term weight loss, which are dietary modification, behavioral modification, supplementation or medication, and the correct type and timing of exercise. Additionally, the understanding that hormone dysregulation, cortisol regulation, stress reduction, hunger and craving control, and improved digestion and elimination all play a critical role in a person's difficulty to both lose weight and maintain their weight loss. These key components are directly addressed by the THINNER Nutrition Strategy program and the all-natural, FDA-registered THINNER medication.

As you progress through this THINNER clinical training course, please know that more detailed instructions for each THINNER phase is contained in both the THINNER Manual and the THINNER app.As previously stated, the THINNER Weight Loss Protocol™ has 2 key components. The first component is the all-natural THINNER medication and the second is the THINNER Nutrition Strategy. Both components are critical.

The THINNER Homeopathic Medication is an all-natural FDA-registered homeopathic medicine that has been classified as an over-the-counter drug. Despite being all-natural, it is regulated by the FDA's Drug Division. It is registered for the purposes of supporting healthy metabolism, improved hormone levels, detoxification, lower stress levels, reduced hunger and cravings, and improved elimination. All ingredients are officially monographed in the H P U S.

The THINNER Nutrition Strategy, which utilizes specific amounts, blends, and ratios of THINNER-approved proteins, THINNER-approved complex carbohydrates, and THINNER-approved fruits along with the implementation of daily intermittent fasting, which has received support for its general health and weight loss benefits. THINNER is not a no-carb program. The intermittent fasting component of the program is accomplished by the person not eating breakfast. Not consuming breakfast was chosen due to the fact that it allows the person to spend the majority of their daily fasting while they sleep. This makes it much easier to accomplish. Additionally, the THINNER eating program teaches participants how to make food choices that aid in the promotion of long-term weight reduction by educating participants on behavioral habits related to food purchases, meal preparation, and food consumption. Additionally, participants are shown how to shop for, prepare, and consume foods that promote long-term weight loss and improved health. The goal, as stated above, is to teach a person an easy-to-understand and easy-to-follow way of actually learning what and how much they should eat.

The THINNER program is 42 days in duration and consists of 4 phases. Across all phases, the average woman can anticipate losing 20 to 27 pounds, and the average man can anticipate losing 25 to 35 pounds over the course of the 6 week protocol.

Phase 1 is called the THINNER Metabolic Activation Phase and is days 1 and 2. The goal of the Metabolic Activation phase is to spike the person's dietary thermogenesis and metabolism by having them consume more food than they typically consume while providing the THINNER medication time to take effect in their body over the course of 24-36 hours depending on the individual. In order to process the increased food intake, the body has to raise its metabolism. During the Metabolic Activation phase, the person is encouraged to consume at least 4,000 calories per day eating continuously throughout the day and not have structured meal times. Ideally, the person will consume approximately 60-70% of their calories in the form of higher fat proteins, which usually minimizes bloating and makes it easier for them to consume the calories. If a person is a diabetic, they should focus their consumption on high-fat proteins, and a person with a history of gallbladder issues or who has had their gallbladder removed should focus on high-calorie carbohydrate consumption. In the United States, the person will take the medication 3 times per day, 7 hours apart. They will administer 1 milliliter each time. Additionally, at the onset of Phase 1, a person will start consuming 80-120 ounces of water per day spread across their waking hours.

Phase 2 is called the Metabolic Thinning phase and is days 3-39. During this phase, the person will start following the THINNER nutrition strategy as instructed in both the THINNER manual and the patient-facing THINNER Instructional video. The THINNER Nutrition Strategy-approved food, as well as THINNER-approved recipes are contained in the THINNER manual and the THINNER app. A typical Metabolic Thinning Phase meal will consist of 5-6 ounces of THINNER-approved lean protein, 5-6 ounces of THINNER-approved complex carbohydrates, and 5-6 ounces of THINNER-approved fruit. Additionally, they will continue administering the THINNER medication at a dose of 1 milliliter 3 times daily. Exercise is not required during the THINNER Metabolic Thinning phase. However, if a patient chooses to exercise, it is important that they limit their exercise to light to moderate walking or yoga for 20 minutes per day during the THINNER Metabolic Thinning Phase. It is common for a person to lose 1-3 pounds daily for the first few days as the body begins flushing itself of systemic inflammation. On or around day 5, weight loss will likely slow (depending on how much inflammation is in the patient's body). Moving forward, weight loss typically averages 0.25 to 0.75 pounds per day, with women being in the 0.25-0.5 pound range and men being in the 0.5-0.75 pound range. Some individuals may find that their weight loss stalls for 1-2 days. If this happens, they should continue with the Metabolic Thinning phase as instructed. If the patient's weight loss stalls for more than 72 hours, you will recommend that they add an additional 1-1.5 ounces of protein at each meal until their weight loss resumes. Guidelines for vegetarian and vegan modifications are included in the THINNER manual and THINNER app.

On day 40, the person will transition into the THINNER Metabolic Transition phase. In this phase, the person will discontinue using the THINNER medication. They will continue following the THINNER nutrition strategy guidelines but will not take the THINNER medication on days 40-42.

Phase 4, the THINNER Metabolic Normalization Phase, with detailed instructions being contained in both the THINNER manual and the THINNER app, begins on day 43. This is where the person who has reached their weight loss goal will lock in their new BMI and BMR. During the normalization phase, the person will begin consuming more traditional amounts of food and, if they choose to discontinue intermittent fasting, will reinstitute breakfast. The person will continue to consume low glycemic index anti-inflammatory foods but in increased quantities. The guidelines for the quantities to be consumed are detailed in both the THINNER program instruction book and in the THINNER app. It is perfectly normal for a person to experience very large swings in their body weight from one day to the next, so do not be alarmed. The swings will get smaller and smaller until it has stabilized. If a person has not reached their weight loss goal and will be doing another round of THINNER, we refer to the THINNER Normalization Phase as a "cycling phase". The person will follow the same guidelines, and the only difference is that they will begin another round of THINNER once their weight has stabilized within 1/2 of a pound for 5-7 days. This will typically take 3-5 weeks. Once this has occurred, the person will be cleared to start their next round of THINNER.

Before we move to the next video, we would like to point out the key parts of this module. The THINNER OTC medication is designed to support appetite control and suppress cravings, stimulate lipolysis and protect lean muscle, stimulate metabolic activity, help regulate cortisol and reduce stress-related eating. It is also designed to help stabilize glucose levels, improve endurance, enhance fatty acid and pyruvate oxidation, and aid in improved digestion, elimination, and mucous membrane support.

THINNER uses 2 key components to address the 4 best practices of successful, long-term weight loss.

The 4 best practices of successful long-term weight loss are dietary modification, behavioral modification, nutritional supplementation, and the correct type and timing of exercise.The 2 key components of the THINNER Weight Loss Protocol are the THINNER Nutrition Strategy and the THINNER medication. THINNER is not a calorie counting program but instead is grounded around healthy amounts, blends, and ratios of lean protein, complex carbohydrates, and fruit to accomplish both weight loss and weight maintenance. The THINNER medication is FDA-registered and does not contain stimulants like caffeine or ephedra. It is imperative that both components be performed in conjunction with each other. The 6-week program is broken into 4 phases... Metabolic Activation, Metabolic Thinning, Metabolic Transition, and Metabolic Normalization. The person who wants to do additional cycles of THINNER MUST have their body

weight stabilize within 0.5 pounds for 5-7 consecutive days.THINNER is not a "no/low carb program"When the video is finished playing, you will be able to progress to "Module 2"

Welcome to module 2 of the Thinner Weight Loss Protocol Clinical Training. In this module, you will learn about the safety guidelines that are built into the Thinner Weight Loss Protocol, as well as the answers to the most commonly asked questions that we receive from newer Thinner offices. Let's start by covering the safety guidelines that are built into the Thinner Weight Loss protocol. After we cover the safety guidelines, we will cover the clinical Thinner Frequently Asked questions.

The Thinner™ Safety Guidelines. The Thinner™ protocol is intended to be healthcare provider-supervised, with follow-up visits scheduled throughout the six-week program. It is recommended that participants meet with their supervising clinician, either virtually or in-person, at least three times: at the start, during week 3, and on day 41 or 42. Each session, lasting approximately 5 to 10 minutes, includes a review of the patient's health, adherence to the protocol, and answering any questions they may have. Additionally, patients are weighed, undergo an 11-point body measurement, and have their blood pressure checked during each visit. For virtual management, patients report this information during telehealth sessions.

All patients are pre-screened for complicating factors, poor prognostic indicators, and contraindications. If any issues are identified during the screening, the clinician will address these concerns with the potential participant. Depending on the severity of the potential clinical implications, individuals with identified complicating factors or poor prognostic indicators may be deemed ineligible for the Thinner protocol. Individuals with contraindications in their medical history, such as active cancer, pregnancy, or breastfeeding, are not eligible to participate in the Thinner protocol.

The Thinner™ protocol follows the Thinner Nutrition Strategy, which focuses on pre-screened, Thinner-approved low glycemic index and anti-inflammatory (L G A I) foods in precise quantities. Foods not included on the approved list should not be consumed.

The Thinner protocol incorporates complex carbohydrates and is not a "protein-only" program. Including complex carbohydrates helps prevent excessive ketosis, hyperuricemia, diuresis, electrolyte loss, and refeeding edema, while also supporting improved muscular endurance.

While undergoing the six-week Thinner protocol, the participant consumes between thirty and thirty-six ounces of nutrient-dense, low glycemic index, and anti-inflammatory foods per day evenly distributed between lunch and dinner.

The Thinner medication is an all-natural, FDA-registered homeopathic OTC formula designed to support weight loss and maintenance. Its benefits include appetite control, stimulation of lipolysis, muscle mass protection during weight loss, increased metabolism, detoxification, cortisol regulation, reduced emotional and stress eating, hunger and craving suppression, glucose stabilization, enhanced endurance, optimization of fatty acid synthesis and oxidation, improved digestion and elimination, diuretic effects, and mucous membrane support.

The Thinner protocol is a six-week program that adheres to scientifically established weight loss standards. It includes healthcare provider supervision, a structured timeline, dietary and behavioral modifications, OTC medication support, and exercise recommendations specific to each phase of the program.

The Thinner protocol provides a “re-feeding” component during which participants transition back to a “normal” diet composed of L G A I foods.

Participants are provided with tailored guidelines, strategies, and tools to support long-term weight loss success. These resources are customized to align with each individual’s BMR, activity level, exercise requirements, and ongoing healthy nutrition habits.

Welcome to the second video for Module 2 of the THINNER Weight Loss Protocol Clinical Training Series. In this video, we will discuss the most common questions we receive from new THINNER offices. We recommend that you get a pen and paper to take notes prior to starting the video… it’s that important!

Why the THINNER Metabolic Activation Phase and is it important?Metabolic Activation is a vital component of the THINNER weight loss protocol. When not done correctly, it is the second most

common cause of reduced weight loss results, after failing to adhere to the protocol. Here's why it matters:Many individuals have consistent eating patterns, consuming a similar calorie count and macronutrient ratio daily, even without monitoring their diet. Over time, the body adjusts to this routine, setting the metabolism to match the intake. The Metabolic Activation phase is specifically designed to increase dietary thermogenesis, which is the body's process of boosting metabolism to handle increased food intake while the THINNER nutritional formula is becoming fully active. This phase encourages eating consistently on higher-calorie foods throughout the day instead of limiting intake to three main meals. This helps ensure that the patient's metabolism is elevated when the THINNER medication becomes active, ensuring the highest possible metabolic rate and optimizing weight loss outcomes.

How come my patient is not losing weight? Sometimes, new THINNER offices ask why a patient isn't losing weight, even when they believe the patient is following the protocol perfectly. Unfortunately, this usually isn't true. If a patient isn't losing weight, it means they are not following the protocol correctly. To be clear, if a patient isn't losing weight, they are not following the THINNER protocol as instructed. If this occurs frequently with a patient, we recommend having the patient take photos of each meal and upload them to the THINNER app. When patients do this, you'll be surprised at how quickly they start or resume losing weight.

My patient's weight loss has slowed or stalled. How do I explain why, and what should I tell them to do? When a patient asks about a weight loss "stall" or slowdown, they often have underlying concerns or questions they may not express. It's essential to address these right away. Reassure them with the following points: Yes, the protocol is still working. No, they haven't done anything wrong. The protocol works—trust the process. Explain that slowing or stalling in weight loss is normal and even expected when following the protocol correctly. The process of burning fat is complex, and a simple analogy can help them understand: The Water Balloon Analogy The Water Balloon: Imagine a balloon filled with water. The water represents fat, and the balloon represents fat cells. Burning Fat: As the body uses energy, it burns fat stored in the "water balloon." This fat is broken down into triglycerides, which are processed by the liver and converted into glycerol and free fatty acids, used as fuel. The end result is that the "water balloon is less full and therefore is smaller." Toxin Elimination: During fat breakdown, toxins stored in the fat are released into the bloodstream and must be excreted. The body uses water to flush these toxins out, which can temporarily cause water retention. Since water weighs more than fat, this retention may result in slowed or stalled weight loss, or even a slight weight gain. Key Points to Reinforce Evaluate progress weekly or over the full 6-week program, not day by day. If the protocol is followed correctly, stalls are a normal part of the process and indicate that the body is working to eliminate fat and toxins effectively. Stalls can occur 1 to 3 times during the protocol, particularly around common milestones like 9 to 11 pounds lost, especially in the first two weeks. Handling Stalls

If a patient reports no weight loss for three consecutive morning weigh-ins have them add 1 to 1.5 ounces of additional THINNER-approved lean protein at each meal or have them do a "protein only day. For a protein only day they will eat 7 to 9 ounces of protein at lunch and 7 to 9 ounces of a different protein at dinner, with no other foods. Evaluate Results: If weight loss occurs the next morning, resume the normal THINNER eating protocol. If no weight loss occurs, repeat a second protein day. After Two Protein Days: If weight loss resumes, return to the normal eating protocol. If no weight loss occurs, follow the normal protocol for two additional days and re-evaluate. Special Considerations For women, a slowdown or stall may commonly occur within 3 to 4 days of the onset of menstruation. This is normal and temporary. By addressing concerns proactively and guiding patients through stalls, you can help them stay motivated and confident in the THINNER protocol's effectiveness.

My patient says that they are hungry; what should they do?

It's important to understand the significant difference between psychological hunger and physical hunger. Psychological hunger can manifest in two ways: Boredom Hunger: This occurs when individuals eat out of boredom, a common habit among those who are overweight. Stomach Growling: Many people mistakenly associate a growling stomach with hunger. However, this is simply bowel noise, not an indication of true hunger. The THINNER nutritional support formula is highly effective at suppressing hunger and cravings, so true physical hunger is rare. However, if a patient does experience physical hunger (hunger pains), the following steps should be taken: Check Water Intake: Ensure the patient is consuming 80 to 120 ounces of water daily. Proper hydration can alleviate hunger sensations. Adjust Food Intake: If water intake is adequate, recommend increasing food consumption at mealtime by adding THINNER-approved free foods. These can be eaten as part of a salad or on their own, but only during mealtimes. A comprehensive list of THINNER-approved free foods are listed in both the THINNER manual and the THINNER app. By addressing these factors, you can help patients manage hunger effectively while staying on track with the THINNER protocol.

My patient is really big. Do I need to modify the nutrition strategy?

Do not modify the THINNER Nutrition Strategy.

My patient asked me about a particular food that's not listed as a THINNER-approved selection. Can they have it? Anything that is 0-calorie is automatically approved. If it's not 0-calorie and not listed in the book, it is not allowed. When speaking with patients, use this simple phrase: "If it's not in the book, it's not allowed—no need to ask." We encourage you to emphasize this to patients. If

you answer questions about one item, they are likely to keep asking about more and more foods. Setting clear boundaries upfront saves time and ensures consistency. Can a patient blend their fruits and vegetables so that they can drink them instead of eating them? Patients are allowed to blend fruits and vegetables, but not juice them. Blending keeps the entire fruit or vegetable intact, including the fiber, whereas juicing removes most of the fiber. Fiber is essential for proper digestion and should not be eliminated.

Why no exercise? Exercise is not recommended during the THINNER protocol for several reasons. The interaction between the THINNER nutrition strategy and medication can increase the risk of exercise-induced injuries. Additionally, individuals who attempt strenuous workouts often find their capacity to maintain exercise routines diminished, with fatigue setting in quickly. Strenuous exercise during this phase has also been shown to result in less weight loss for patients. During the THINNER Metabolic Thinning Phase, approved is limited to light to moderate walking or yoga for 20 minutes. It is important to note that exercise is not a required component of the THINNER program. However, because exercise plays a key role in long-term weight maintenance and overall health, patients are encouraged to resume their previous routines or begin a more intensive program once they transition into the THINNER Metabolic Normalization phase. My patient said that they had spotting or had an unexpected menstrual cycle; why and what should I tell them? Sometimes, female patients may experience spotting or an unexpected menstrual cycle while on the THINNER protocol. This occurs because estrogen is stored in fat cells, and as the body breaks down fat for energy, the released estrogen enters the bloodstream. In some cases, this can trigger spotting or an unexpected menstrual cycle.

My patient reported cramping in their legs; what should I do? Leg cramping during the THINNER protocol is usually caused by a deficiency in trace minerals or low electrolyte levels. To help prevent this, patients are encouraged to use high-quality Pink Himalayan Salt, which is rich in trace minerals. If cramping does occur, an electrolyte replenisher is recommended to address the symptoms. Can my patient take a particular supplement? Any food or supplement that is 0-calorie is automatically approved, with no need to ask for confirmation. If it's 0-calorie, it's approved. My patient is constipated. What should I do or recommend? A decrease in the number and frequency of bowel movements while on the THINNER protocol is relatively common and can occur for several reasons. The most significant factor is the dietary change, which includes a substantial reduction in food volume—less food in means less waste out. Additionally, the foods in the protocol are lean (with less fat for lubrication), clean, nutrient-dense, and free of fillers, resulting in less waste passing through the bowels. If a patient mentions they haven't had a bowel movement for several days, there is no need for concern, even if they were accustomed to more frequent movements in the past. Ask if they are experiencing any abdominal discomfort and ensure they are drinking 80 to 120 ounces of water daily— not just liquids, but specifically water. If needed, you can recommend a sugar-free fiber supplement or an OTC stool softener.

For patients with a history of chronic constipation, it's advisable to start a sugar-free fiber supplement on Day 1 of the THINNER protocol and continue taking it daily—not just throughout the protocol but as part of their ongoing routine after completing THINNER.

My patient takes medication. Can they do the protocol? Given the dilute homeopathic nature of these ingredients, significant interactions with conventional drugs are not highly likely. My patient has hypothyroidism and takes medication for it. Can they do the protocol, and will the protocol work? THINNER is highly effective for patients with hypothyroidism, who can expect results comparable to those without the condition. In some cases, patients with hypothyroidism have even experienced improvements in their condition, leading their medical doctor to reduce their medication dosage. Is it ok for a patient who takes antidepressants or anti-anxiety medication to do THINNER? As mentioned before, given the dilute homeopathic nature of these ingredients, significant interactions with conventional drugs are not highly likely. It has been noted that on occasion, a patient who has been on serotonin reuptake inhibitors (SSRIs) for a prolonged period of time may have slower than normal weight loss and may lose 3 to 5 pounds less than normal. Is the THINNER protocol ok for a woman who has had a hysterectomy? A hysterectomy is not a contraindication for the THINNER protocol. However, it's important to let patients know that if they are on hormone replacement therapy (HRT), even bioidentical HRT, they may lose 3 to 5 pounds less than those not on HRT. Despite this, they can still expect almost the same amount of inch loss. My patient has diabetes. Can they do THINNER? Yes, THINNER is safe for individuals with diabetes and can even improve their condition. Weight loss is highly beneficial for diabetics, and many patients see their blood sugar levels lower and stabilize while on the protocol. However, it's important to note that diabetic patients taking medication may need their medication dosage adjusted by the medical professional that precscribed the medication. When starting the protocol, set expectations by explaining that as they lose weight and improve their diet, they may experience symptoms of low blood sugar or hypoglycemia. This happens because their blood sugar is naturally decreasing. If they notice symptoms of low blood sugar, they should contact their doctor to adjust their medication dosage. Occasionally, patients may experience symptoms of hypoglycemia in the morning. If this occurs, recommend that they begin eating 3 ounces of fruit in the morning to help stabilize their blood sugar.

My patient said they have been having headaches since starting THINNER. Why? Some people may experience mild headaches during the first week of the THINNER protocol. This typically happens for two reasons: the body adjusting to the reduced intake of simple carbohydrates and the detoxification process. If your patient experiences an ongoing nagging dull headache, ensure that they are drinking the recommended amount of water each day. They may also benefit from taking a high-quality electrolyte supplement or receiving an IV hydration treatment with electrolytes to help alleviate the discomfort. My patient has a rash or is itching. Is it the medication? Any rash, itching, or allergic-type reaction is typically due to detoxification, or the patient has introduced something into their diet that they are sensitive to and not the THINNER Protocol medication. My patient is

sick. Should they stop the protocol? There is no need for a patient to stop the protocol due to a routine cold or sickness.

My patient was just put on an antibiotic. Should they stop the protocol? A patient's use of an antibiotic does not have any impact on the effectiveness of the THINNER protocol, and they do not need to stop. My patient was just put on a steroid. Should they stop the protocol? Patients prescribed steroids should be given the option to pause the THINNER protocol. This is because steroids often cause water retention and increased hunger, which can make it challenging for some individuals to adhere to the protocol effectively.. If my patient needs to stop the protocol, how long do they need to wait to restart? If a patient needs to stop the THINNER protocol, they should wait at least 10 days before restarting. When they do restart, they must begin again from Day 1, starting with the Metabolic Activation phase.

Why did my patient run out of the THINNER medication, and what should I do? If a patient reports running out of THINNER medication, it is typically for one of three reasons: they spilled it, they are taking more than the recommended dose, or they are sharing it with someone else. IIn cases where a patient runs out prematurely, it is recommended to sell them a new bottle. The suggested price for the medication alone is $450 to $500. Can A patient Drink Diet Soda? Yes, the patient can drink diet soda. ANy beverage that is 0-calorie is THINNER approved

Why Can't A Person Eat 3 Meals Per Day? This concept is rooted in the proven benefits of intermittent fasting, which can positively influence the hypothalamus—a key regulator of metabolic processes, hunger, and energy balance. The hypothalamus produces hormones that control appetite and stress responses. Intermittent fasting activates pathways in the hypothalamus that respond to nutrient availability, enhancing sensitivity to hormones like insulin and leptin, which are essential for managing hunger and energy use. It also stimulates the production of brain-derived neurotrophic factor (BDNF), a protein linked to improved brain function and neuroprotection.

Is there a vegetarian plan? Yes, the information regarding vegetarians and the THINNER protocol is available in both the THINNER Protocol manual and the THINNER app. However, vegetarians often face the most challenges when following the protocol, as it may require more careful planning and adjustments to meet the nutritional requirements. For those who are 'mostly' vegetarians but occasionally eat seafood, they can substitute any THINNER-approved seafood in place of poultry, pork, or beef. This flexibility can make the protocol more manageable for individuals with such dietary preferences. Can a patient have eggs? We do not recommend eggs, which is why they are not included as a THINNER-approved food.

Can a patient who has had a gastric bypass, lap band, or any other weight loss surgery do the protocol? Yes, individuals who have undergone any type of weight loss surgery can follow the THINNER protocol. However, some may require modifications to the eating plan. Due to the restricted stomach capacity from surgery, these patients may need to spread the required meal quantities over a period of 2 to 3 hours instead of consuming them in one sitting. The Metabolic Activation phase can be particularly challenging for these individuals. They should be encouraged to do their best and focus on consuming higher calorie-dense foods to meet the protocol's requirements effectively. Does the THINNER protocol affect birth control? Given the dilute homeopathic nature of these ingredients, significant interactions with conventional drugs are not highly likely.

My patient said that they seem to be losing hair. What should I tell them? It's not unusual, especially for women, to notice some increased hair shedding during significant weight loss. If this happens while on THINNER, it's usually because they're not consuming enough protein, as outlined in the THINNER Nutrition Strategy. Simply have them increase their protein intake by 1 ounce at each meal. The hair loss is temporary and will resolve as they adjust. My patient only wants to lose 15 to 20 pounds. Can they do the THINNER protocol for less than 42 days? Yes, a patient can follow the THINNER protocol for less than 42 days. However, to help establish new set points for BMI and BMR, they need to stay on the protocol for at least 21 days before transitioning to the THINNER Transition phase. If the patient only needs to lose a quick 10 to 15 pounds, We recommend putting them on the THINNER protocol. The shelf-life of the THINNER medication is 2 years. They can store it and use it at a later date.

Can THINNER be billed to a patient's insurance? The THINNER Weight Loss Protocol is not designed to be submitted to a patient's health insurance and should not be submitted. However, THINNER typically qualifies for use with most health savings accounts (HSAs) and flexible spending accounts (FSAs). Patients across the United States are often reimbursed for the cost of the THINNER protocol through their HSA or FSA.

If your office uses 3D body imaging equipment to take body measurements, you can skip this module.

Welcome to Module 3 of the THINNER clinical training. In this module, we will teach you the best practices for accurately taking THINNER patient body measurements. For patients being supervised remotely, it is recommended they have a significant other, friend, or co-worker assist

with their measurements, as it is nearly impossible to take accurate body measurements on their own. Additionally, it's best to have the same person take the measurements each time for consistency.

Neck: The patient should stand comfortably while the measurement is taken around the neck.

Shoulders: With the patient standing comfortably and arms at their sides, wrap the tape measure around the shoulders, approximately 1 to 2 inches below where the shoulder transitions into the upper arm.

Chest: Have the patient raise their arms to the side. Wrap the tape measure around the chest at the nipple line, then have the patient lower their arms, stand comfortably, hold their breath, and take the measurement.

Stomach: With the patient raising their arms slightly, wrap the tape measure around the stomach at the navel level. Once the tape is positioned, have the patient lower their arms, stand comfortably, and take the measurement.

Hips: Raise the patient's arms slightly and locate the bony area on the outer hips. Wrap the tape around this area and take the measurement while the patient stands comfortably.

Right Bicep: Have the patient stand comfortably with arms at their sides. Wrap the tape around the middle of the upper arm and take the measurement. Left Bicep: Follow the same procedure as the right bicep.

Right Thigh: Have the patient stand comfortably with arms at their sides. Place the tape around the middle of the thigh and take the measurement. Left Thigh: Follow the same procedure as the right thigh.

Right Calf: With the patient standing comfortably, place the tape around the mid-calf and take the measurement. Left Calf: Follow the same procedure as the right calf. By following these steps and

ensuring consistency in the process, you can achieve accurate and reliable body measurements for your THINNER patients.

Welcome to Module 4 of the THINNER Clinical training series, where we will discuss the most commonly asked questions you will get from your patients and what the correct answer is. Let's get started.

What is THINNER, and how does it work? The THINNER Weight Loss Protocol is built on a two-pillar approach that supports weight loss and incorporates four key principles for sustainable, long-term weight maintenance. The two pillars of the program are the THINNER Homeopathic OTC Medication and the THINNER Weight Loss & Weight Maintenance Nutrition Strategy. The THINNER Homeopathic OTC Medication is an all-natural, FDA-registered homeopathic medicine designed to support healthy weight loss by balancing metabolism, reducing stress, curbing hunger and cravings, aiding detoxification, and improving elimination. It plays a crucial role in a comprehensive weight management plan. The THINNER Weight Loss & Weight Maintenance Nutrition Strategy is a science-based approach focusing on specific portions and combinations of THINNER-approved low glycemic index and anti-inflammatory foods. By emphasizing these carefully selected foods, the strategy promotes sustainable weight loss, reduces inflammation, and supports overall health, making it ideal for long-term weight management. The THINNER program is clinically proven to deliver significant weight loss faster and at a lower cost than GLP or semaglutide, offering an effective and affordable alternative.

Where is THINNER manufactured, and what are the quality control guidelines? All THINNER products are manufactured in the United States in FDA-registered, cGMP-certified, and state-licensed facilities that undergo regular FDA inspections. After production, all medications and supplements undergo rigorous quantitative testing to ensure they meet strict standards for product specifications, potency, and safety before being released for sale. THINNER products are produced in compliance with Good Manufacturing Practices (GMP) and the Code of Federal Regulations (CFRs), ensuring the highest quality and safety standards.

How much weight can I expect to lose through the THINNER protocol? Weight loss results on the 42-day THINNER protocol vary from person to person, but most people will lose 20 to 35 pounds when following the 6 week protocol. Like every other weight loss treatment, men will typically lose weight faster than women. The patient's results depend on several factors, including the amount of excess weight, the total weight that needs to be lost, genetics, and an individual's commitment to following the THINNER protocol as instructed. Everyone's journey is unique.

Will I get hungry? It's common to experience mild psychological hunger during the first few days of the THINNER protocol, but this sensation typically subsides within 2 to 4 days of starting the THINNER Metabolic Thinning Phase. The THINNER medication is formulated to help control hunger and cravings, making true physical hunger relatively uncommon once your body adapts to the new eating patterns.

Does the weight loss slow down after the first two weeks? It's common to experience significant weight loss during the first week of the THINNER protocol, followed by a gradual slowing. This doesn't mean weight loss has stopped. In fact, over the course of the protocol the average woman will experience an average wieght loss of 1/4 to 1/2 pound per day and men will average 1/2 to 1 pound per day over the course of the 6 weeks when following the protocol. Additionally, throughout the protocol, inches are typically being lost continuously, even when the scale shows slower progress.

Will THINNER interfere with any medications I am currently taking? What about birth control? Given the dilute homeopathic nature of these ingredients, significant interactions with conventional drugs are not highly likely. Should you have any specific concerns, please consult your healthcare practitioner.

Will I experience any changes in my menstrual cycle by taking THINNER? None of the ingredients in the THINNER protocol are known to directly affect the menstrual cycle. Additionally, THINNER will

not impact your ability to become pregnant or increase your chances of pregnancy. However, it's recommended to avoid starting the protocol within 5 to 6 days of your menstrual cycle. If you are taking fertility medications, we advise against beginning the THINNER protocol. Occasionally, some women may experience spotting or an unexpected menstrual cycle. This occurs because the estrogen stored in fat cells is released into the bloodstream as the fat cells are being emptied to provide energy.

Can I take THINNER while pregnant? A woman is prohibited from participating in the THINNER protocol if she is pregnant. If you become pregnant during the THINNER protocol, stop taking THINNER immediately and consult your physician. Is it ok to skip a meal or protein at one of my meals? Always eat all your meals and consume the full portions of food allotted to you, especially the protein. It's also important not to shift portions from one meal to another—stick to the plan as outlined for the best results.

Can I exercise daily? Refer to the THINNER Manual or THINNER app for the THINNER Metabolic Thinning Phase approved exercises.

Is it normal to get a headache during the first week? Light headaches during the first week are common for some people. This is usually due to two factors: withdrawal from simple carbohydrates and the body's natural detoxification process. To alleviate headaches, make sure you're drinking the recommended amount of water daily. You may also consider using a high-quality electrolyte supplement or receiving an IV hydration treatment with electrolytes.

Can I stay on THINNER for more than 42 days if I want to keep losing more weight? No. After 42 days, you need to enter the Metabolic Normalization phase for a minimum of 28 days or until your body weight has stabilized within 0.5 pounds of itself for 5 to 7 consecutive days.

Can I drink alcohol while taking THINNER? It's strongly recommended that you not consume alcohol while on THINNER.

Welcome to module 5 of the THINNER Weight Loss Program Clinical Training module. In this module, we will discuss the concepts and wisdom of offering 3 unique THINNER packages to your weight loss clients. Please note that as a THINNER Protocol office you receive a 1 on 1 session with our team so we can help you build out bespoke packages and pricing models to present to your patients.. While not required, it is strongly encouraged that you develop 3 different packaged offerings for THINNER programs in your practice. The benefit of offering packages has been proven time and time again.

Studies have shown that when a person is offered 2 options, 80% of people will purchase the least expensive package and 20% will purchase the higher priced option. However, when a person is presented with 3 options, 60% of people chose the middle option, 20% chose the lowest priced option, and 20% chose the highest priced option. The reason for this is that when offering 1 option, the person is forced to be a "yes or no" consumer. When a person is presented with 2 options, the large majority of people automatically become "bargain shoppers." This is why 80% of people will choose the lower priced option when presented with 2 choices. When you offer 3 options, the average consumer is transitioned from a "bargain shopper" to a "value buyer." This is reflected in the 60% of individuals choosing the middle option.

The first step in the process is to determine what you will charge for your basic THINNER package and what it contains. Keep in mind that the large majority of THINNER offices charge at least $1000 for their base package. Your base package should be the least expensive and include the least amount of products and services.

As you build out your 2nd and 3rd package offers, each one should include additional products, procedures, and build value. Remember, 60% of purchasers will choose option 2, so design the next package wisely.

It is up to you regarding what to include, but adding in a few follow-up maintenance visits, other services, and for the top package, the "Lose 20 Pound Guarantee," are great ways to build value. Additionally, you can add in things like priority access to the doctor or staff, etc. Use this same process when developing option 3.

Also, when setting your prices, do not underestimate the value of this program as the results are significantly faster than GLP1.

As mentioned at the beginning of this video, as a THINNER office, you receive a 1 on 1 session with our team so we can help you build out bespoke packages and pricing models to present to your patients at no cost. We will help you build them out based on your market, demographics, and other services that your practice offers.